Evidence-Based Assessments for Brain-Based Informed Mental Health Treatment

What Are We Assessing?

Traditional mental health assessments often overlook brain function, despite the complexity and overlap of many mental health conditions. To gain a more comprehensive understanding, brain-based informed mental health clinicians can integrate additional assessments, such as IEPs, neuropsychological evaluations, and developmental evaluations (OT, PT, SLP), into their treatment approach. As they transition into a brain-based informed mental health practice, they can learn how to interpret these reports meaningfully and connect results to behavioral support strategies. Additionally, for those without formal evaluations, alternative methods can help assess brain function more effectively.

Prior to Initial Assessment

Requesting and reviewing patients' existing documents can enhance the assessment process. Consider obtaining and analyzing the following:

- Previous neuropsychological assessments
- Previous Occupational Therapy (OT) evaluations and treatment plans
- Previous Speech and Language evaluations and treatment plans
- Previous school evaluations
- Individualized Education Plan (IEP)
- Previous mental health evaluations and treatment plans
- Medical and medication records
- Client/Patient interviews regarding prenatal and birth history, including assessment of prenatal alcohol exposure (PAE), prenatal drug exposure (PDE), or other potential factors

Assessments to Consider

An effective assessment process includes screening for prenatal alcohol exposure. Familiarizing oneself with language that reduces stigma is essential when assessing for PAE. Refer to the following guides:

- **Prenatal Alcohol Exposure Assessment**
- **Language and Stigma Guide**

Experts recommend a **comprehensive neuropsychological evaluation** to assess strengths, deficits, co-occurring conditions, and early childhood trauma. If such an evaluation has not been previously administered, clinicians can rely on their observations of an individual's conceptual, social, and practical skills through informal assessments.

Two informal assessments include:

- **Vanessa Spiller's Ability Wheel**
- **FASCETS Neurobehavioral Exploration Tool** (Clinicians must be trained in the FASCETS Neurobehavioral Model for appropriate application and can access more information about the training at **FASCETS Center for Neurodiversity**)

If informal assessments indicate additional needs outside the clinician's scope, referrals should be made accordingly.

Common Evidence-Based Assessments for Brain-Based Challenges

The table below outlines common assessments, including who can administer them, where results may already be available, and appropriate referral sources.

Primary Characteristic	Evidenced-Based Assessment	Age Range	Who Can Administer/Where to Find Results
Memory	1. Mini Mental Status Exam for Children 2. Children's Memory Scale 3. WRAML-2 4. NEPSY-II	1. 3-17 2. 5-16 3. 5-90 4. 7-12	1. Any trained professional 2. Neuropsychologist / Neuropsychology report

			3. Neuropsychology or School Psychology report 4. Neuropsychology or School Psychology report
Development	1. Bayley Scale of Infant Development (4th ed.) 2. 2. Vineland Adaptive Behavior Scale	1. 1 month-43 months 2. Birth-90	1. Trained individual 2. Masters-level professional in psychology, education, or social work
Sensory Issues	1. Sensory Profile	Infant: birth-6 months Toddler: 7-35 months Child: 3-14 years	Caregivers and teachers (rating forms) / Occupational Therapist
Processing Speed & Thought Process	1. WPPSI 2. WISC 3. WAIS 4. IQ Tests	1. 2.6-7.7 years 2. 6-16 years 3. 16-90 years	Psychologists / Psychological & School Psychological reports
Speech/Language	1. CASL-2	1. 3-21 years old	Speech-Language Pathologist / Speech and Language Evaluations (school or private)
Auditory Processing	1. Dichotic listening tests 2. Speech-in-noise tests 3. Auditory discrimination tests 4. Monaural low-redundancy tests 5. Rapid alternating speech tests 6. Pitch pattern sequence tests 7. SCAN (test for APD)	Varies	Speech-Language Therapists / Audiologists / Speech-Language or Audiologist Reports

Learning Style	1. VARK Assessment 2. Learning Style Inventory for Children 3. Observation during play & daily activities	1. 12+ 2. Elementary school 3. Under 6	1. Self-administered questionnaire 2. Teachers, school counselors, educational psychologists 3. Caregivers, parents, teachers
Behavioral Functioning	1. BASC-3 2. BRIEF	1. 2-21 years 2. 5-18 years	1. Psychologists, trained individuals (Teachers & Parents can complete rating forms) 2. Parent questionnaire
Attention/ Executive Functioning	1. Conners Comprehensive Behavior Rating Scale 2. Conners Parent & Teacher Rating Scale 3. Conners Early Childhood	1. 6-18 2. 6-18 3. 2-6	1. Trained mental health professionals 2. Parents & teachers (rating forms) 3. Trained professionals, teachers, parent rating forms
Trauma	CATS (Child and Adolescent Trauma Screen)	1. 3-6 years 2. 7-17 years	1. Caregiver report 2. Caregiver & self-report

Other Considerations

- Assess the **cognitive capacity** of the support system (parent/caregiver/support person)
- Evaluate the **learning style** of the support system
- Assess the **level of stressors** within the environment and support system
- Identify available **resources and services** for both the client and their support system

By integrating these evidence-based assessments into mental health treatment, clinicians can better understand brain function and provide targeted, individualized support to enhance patient outcomes.